

A Pilot Examination of the Use of Goal Attainment Scaling with a Physically Disabled Summer Camp Population

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This study sought to explore the usefulness of Goal Attainment Scaling (GAS) as a means of determining how well the goals of physically handicapped adolescents and adults were achieved in a recreational summer camp program. Goal Attainment methodology was used with 17 adults and 15 adolescent physically disabled individuals who were participating in 12-day programs at Camp Courage, located near the Minneapolis-St. Paul metropolitan area. Graduate students trained in this technique interviewed the campers about their goals in four areas: socializing, self-care, emotional well-being, and an optional area. Overall, the goals were met at the "expected" level. There were few differences in either the content of the goals, the level of achievement between adults and adolescents or between first-time and returning campers. More goals were identified in the socializing and optional areas than in self-care or emotional well-being. Goal achievement was comparable across the four categories. The advantages and difficulties involved in using GAS methodology in this type of setting were also discussed.

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Methods

Subjects

The study was conducted during June

which he or she would like to make some change (for example, losing weight or learning to swim) and then decide how much he or she can reasonably expect to achieve in the amount of time available. (Occasionally, in the face of a potentially limiting condition, the goal may relate

The short-term, residential summer camp experience for disabled individuals provides an ideal setting in which to teach new skills and behaviors and to eliminate maladaptive ones. Allen (1973) has outlined several features of this environment which uniquely suit such purposes. First, camp routines allow for large numbers of social interactions. Secondly, the novelty of the camp experience can function as an incentive for behavior change. Finally, the intensity of the experience allows campers the opportunity to try out new behaviors which they might not be able to attempt in other settings such as school or home.

Though the therapeutic potential for such camps is high, very little evaluative research in such settings has been published. Due to continuing needs for accountability, however, it is becoming necessary to search for methods of assessing the effectiveness of the camp experience in its ability to achieve demonstrable goals. Byassee and Tamberino (1975) point out the lack of useful data resulting from global evaluative measures such as personality inventories. These authors emphasize the importance of implementing instruments which can detect changes due to short-term treatment and are sensitive to specific behavior change.

The present study described one such attempt to implement Goal Attainment Scaling (GAS) as an evaluative instrument for the summer camp setting. Originally designed by Kiresuk and Sherman (1968), GAS has been widely used in the evaluation of mental health programs (Kiresuk 1973; Kaplan and Smith 1977). It has also been implemented in a wide variety of other settings including school counseling, rehabilitation programs, occupational therapy, and special education. Since its original publication, GAS has become so widespread that authors such as Cytrynbaum et al. (1979) have referred to GAS as the most popular

outcome evaluation technique in the human services.

Research over the past decade indicates a moderate reliability across raters for GAS scales, with correlations ranging from .51 to .85 (Calsyn and Davidson 1978). Because validity studies comparing GAS to other measures of client change, such as the MMPI, range only from .19 to .50, Kettlewell et al. (1977) recommend using GAS in combination with other techniques in order to obtain a full measure of program effectiveness.

The advantages of implementing GAS in the summer camp setting lie in its adaptability to various types of campers, its relevance to the specific needs of these campers, and its ability to aid in organizing and numerically recording the effectiveness of such short-term programs. Gold et al. (1977) also emphasize the increase in client motivation when such goal setting programs are implemented. In fact, 67% of the GAS users surveyed by Calsyn et al. (1977) used the GAS as a therapeutic tool rather than as an evaluative measure.

GAS has been applied at a preliminary level to therapeutic summer camping programs using psychiatric populations (Byassee and Tamberino 1975). These authors' data suggest an overall positive evaluation of these camp programs. These findings are in contrast to previous studies where global measures have failed to indicate camper progress. Such results underscore the need for evaluation of the summer camp experience through individually tailored measures such as GAS.

The following study explores further the ability of GAS to meet such a need by using it, first, to determine what benefits adolescent and adult physically disabled campers might attempt to derive from their stay at camp and, secondly, to investigate the effectiveness of the

camp program their objectives

Methods

Subjects

The study and July 1980 recreational summer disabled and paired children Lake, Minnesota area of Minnesota participants were campers from 17 physically the adult sessions all first-time sessions (except too severe a commit interview and 10 adolescents who were self group. The comprised of sixteen while the adult eleven female from 25 to 4 years. The 13 to 17 with Cerebral palsy disability with seven adults disabilities in spina bifida, ties, blindness injuries.

Procedure

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Methods

Subjects

The study was conducted during June
and July 1980 at Camp Courage, a re-
creational summer camp for physically
disabled and speech and hearing im-
paired children and adults, in Maple
Lake, Minnesota, near the metropolitan
area of Minneapolis-St. Paul. The par-
ticipants were 15 physically disabled
campers from the adolescent session and
17 physically disabled individuals from
the adult session. This sample included
all first-time campers from these two
sessions (except one person who had
too severe a cognitive impairment to per-
mit interviewing) along with 13 adult
and 10 adolescent returning campers
who were selected at random from each
group. The adolescent group was com-
prised of six males and nine females,
while the adults numbered six males and
eleven females. The adults ranged in age
from 25 to 46, with a mean age of 35.29
years. The adolescents' age range was
13 to 17 with a mean of 14.37 years.
Cerebral palsy was the most common
disability with nine adolescent and
seven adults having that diagnosis. Other
disabilities included muscular dystrophy,
spina bifida, other orthopedic disabili-
ties, blindness, and brain diseases and
injuries.

Procedure

The interviewers for this study were
six graduate students in Counseling
Psychology at the University of Min-
nesota who were trained during two
two-hour sessions to use the GAS meth-
odology. In this system for defining
individualized goals in behavioral terms
and for measuring how well those goals
have been met, each participant identifies
several areas (usually three to five) in

which he or she would like to make some
change (for example, losing weight or
learning to swim) and then decides how
much he or she can reasonably expect to
achieve in the amount of time available.
(Occasionally, in the face of a potentially
worsening condition, the goal may relate
to maintenance rather than positive
change). That prediction is then recorded
as the individual's "expected" level of
achievement on the GAS form, which is
called the "Goal Attainment Follow-Up
Guide." On either side of the "expected"
level other possible outcomes are re-
corded. Two are lower levels of achieve-
ment ("somewhat less than expected"
and "much less than expected") while
the others are higher ("somewhat more
than expected" and "much more than
expected").

An observable outcome for each of
these levels as well as for the "expected"
level is specified at the point of intake
into the activity or program in which the
individual's progress is to be evaluated.
This usually takes place in an interview
during which the client and a trained
interviewer jointly determine the out-
comes to be recorded at each level. For
example, a camper who hopes to learn to
swim might decide that her goal is to
swim one length of the pool by the end
of the 12-day session. That becomes
her "expected" goal. However, she might
admit to the possibility that even if she
were to try hard to learn, she might still
leave camp without knowing how to
swim at all; and that would constitute
her "much less than expected" level.
In between, she might learn to swim a
few strokes or to float for at least 30
seconds; and this could be her "some-
what less than expected" level. On the
other side of the expected level, "more
than expected" might constitute learning
to swim two lengths of the pool; and
"much more than expected" might be
learning to swim three lengths or more.

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e score is 0. "Somewhat
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ected" is +2. Similarly,

for home, the interviewers returned to
the camp to talk with the subjects about
their experiences and to determine how
well their goals had been met. In most

Emotional well-being goals were even
more difficult for the campers to identify
than the self-care objectives. The investi-
gators had intended to focus on such

It should be evident that the GAS methodology is extremely flexible. For example, a camper who is already an adept swimmer might also set a swimming goal, but this goal might be defined in terms of racing times or learning various strokes or swimming much longer distances. The camper's goal could also focus on some other skill, such as ceramics or horseback-riding. Behavioral objectives specify an accomplishment or something that the individual is going to be able to achieve. This is also known as an "outcome measure."

A subtly different kind of goal is a "process measure." This goal focuses on the experience rather than the achievement. For example, rather than indicating how much she expects her skill to improve, the swimmer might state that she simply wants to go swimming at least five times during the camping session. Process goals are less meaningful under the GAS system, but they can nevertheless be used when the camper cannot specify outcome measures.

At the point of follow-up, another interview takes place so that the interviewer and the client can jointly determine to what extent the goals have been met. Outcomes are recorded by placing a check mark (✓) at the appropriate level on the GAS Follow-Up Guide. Each goal is then assigned a score. With GAS, if

$$* T = 50 + \frac{10 \sum w_i x_i}{\sqrt{(1-p) \sum w_i^2 + p (\sum w_i)^2}}$$

The variable x_i is the numerical value for the outcome on the i th goal attainment scale, w_i is the relative weight for the i th scale (GAS also provides an option for selecting a set of weights for the goals that reflect an appraisal of the relative value of each goal. Weights were not assigned in this study; and for purposes of using the formula, w was set equal to 1 for all goals.), and p is the expected overall intercorrelation among goal scores. (In most cases p is assumed to equal 0.3.)

a person achieves a goal at the "expected" level, the score is 0. "Somewhat more than expected" is +1, and "much more than expected" is +2. Similarly, "somewhat less than expected" is -1, while "much less than expected" is -2.

GAS methodology also provides a formula* by which the individual's several outcomes can be combined and translated into an overall "Gas score" (Kiresuk and Sherman 1968). Applying this formula, an individual whose outcomes average "0" (at the "expected" level) would have a GAS score of 50, while a participant whose outcomes had an average of "+1" (at the "somewhat more than expected" level) would receive a GAS score of 60. Similarly, an average of -1 ("somewhat less than expected" level) would be converted to a GAS score of 40, +2 to 70, and -2 to 30.

An initial interview took place with each participant shortly after he or she arrived at camp. Each person was asked to define goals in three specific areas—self-care skills, socializing, and emotional well-being—and in a fourth area of the camper's own choosing, referred to as "optional." The interviewer and the participant also decided at the time of the goal-setting how each outcome would be measured. In some cases camper self-report seemed appropriate and adequate. However, daily logs of activities or social experiences were kept in a number of instances. The use of counselors as record-keepers or raters was also considered but was generally avoided for fear that such a role might interfere with the counselor's function as a companion and friend. In all cases campers were assured that the goals set would be kept completely confidential. Campers were given a copy of the GAS Follow-up Guide to keep, and they could choose whether or not to share it with anyone else, including staff members.

Ten days later, on the day before most campers were scheduled to leave

for home, the campers' expectations were well thought out, and they were able to keep their goals, and they were able to

Results Types

Can reported goals be used to predict activity, emotional, or social behavior? The

The seeing goals focus on initiating longer different time

In providing information and identifying areas of work they were able to help the campers to

They were able to achieve their goals

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chieves a goal at the "expected" level, the score is 0. "Somewhat expected" is +1, and "much expected" is +2. Similarly, "less than expected" is -1, "much less than expected" is -2. The methodology also provides a way in which the individual's scores can be combined and converted to an overall "Gas score" (see Sherman 1968). Applying the formula, an individual whose average "0" (at the "expected" level) would have a GAS score of 60. A participant whose average of "+1" (at the "more than expected" level) would have a GAS score of 70. Similarly, a participant whose average of "-1" ("somewhat less than expected") would be converted to a GAS score of 50.

The interview took place with the camper shortly after he or she returned to camp. Each person was asked to identify goals in three specific areas—socializing, emotional well-being—and in a fourth area of their own choosing, referred to as "optional." The interviewer and the camper also decided at the time of the interview how each outcome would be scored. In some cases campers seemed appropriate and wever, daily logs of activities and experiences were kept in a notebook. The use of counselors or rater's was avoided but was generally avoided such a role might interfere with the counselor's function as a confidant. In all cases campers were assured that the goals set would be kept confidential. Campers received a copy of the GAS Follow-up form, and they could choose to share it with anyone on the staff members.

Later, on the day before the campers were scheduled to leave

for home, the interviewers returned to the camp to talk with the subjects about their experiences and to determine how well their goals had been met. In most cases, when campers had initially agreed to keep records of their progress toward goals, they followed through and brought them to the follow-up interview session.

Results

Types of Goals Set

Campers and interviewers both reported that it was much easier to set goals in the areas of socializing and activities (which tended to fill the optional category) than in the self-care or emotional well-being areas. A summary of the goals selected by the campers is shown in Table 1.

The hope of meeting new friends and seeing old ones dominated the social goals of all campers. A few participants focused on developing their skills at initiating conversations or on spending longer amounts of time socializing. Few differences were apparent between first-time and returning campers.

Interviewers reported that they had to provide much encouragement to campers in order to arrive at self-care goals and that even then only half could identify an activity to work on in that area. Some subjects said that they had come to camp to have fun and not to work on such tasks, while others said they were already doing as much as possible in self-care areas. However, one or two individuals commented at follow-up that these goals had been particularly helpful to them because the process had led them to make progress on some important skill.

The self-care goals that were defined were quite diverse. Three individuals aimed to improve some aspect of dressing, two worked on transfers, and two tried to lose weight. Each of the other goals was specified by only one person.

Emotional well-being goals were even more difficult for the campers to identify than the self-care objectives. The investigators had intended to focus on such things as improvement in self-esteem, reduction in tension, increased feelings of independence, or competence. These expectations were not met, however. More than two-thirds of the campers were unable to identify any goals in this area, and several of the ones that were listed seemed to belong instead in the socializing or self-care categories. On the other hand, one person did strive to think more positively about his disability; one attempted to do nice things for other people, one tried to accept group decisions gracefully, and one was determined to avoid feeling homesick.

The optional goals generally referred to the number of times the camper would engage in various activities ("process" goals, as explained above). Most often the campers aspired to take part in a substantial number of varied activities. Among the individual areas, swimming and horseback-riding seemed the most popular.

Goal Attainment

In addition to identifying the range of objectives that campers might pursue, a second purpose of this study was to determine how well such goals were being met. The overall GAS mean score for the adult campers was 50.46, and for the adolescents it was 47.86. (Student's *t* test of mean differences did not yield any significant difference between groups). These scores confirm that, overall, the goals being set were realistic and that they were met during the camping experience.

The goal achievement of new campers in the four areas was contrasted with that of returning campers using Student's *t* test of mean differences. Among the adolescents, there was no significant dif-

Table 1.
Summary of Goals Chosen by Adolescent and Adult Campers

GOALS	CAMPER GROUP			
	Adults		Adolescents	
	1st Time (N=4)	Returning (N=13)	1st Time (N=5)	Returning (N=10)
Social				
Making new friends	2	5	3	6
Renewing acquaintance with old friends	—	4	—	2
Initiating conversations	2	2	—	—
Spending time socializing	—	1	—	—
Getting to know names of cabin-mates	—	1	1	2
Meeting female campers	—	—	1	—
None	—	—	—	—
Self-Care				
Improved hair-grooming	—	2	—	—
Brushing teeth	—	1	—	—
Losing weight	—	2	—	1
Washing self in shower	—	1	—	—
Improved self-feeding and drinking	1	—	1	1
Refusing unnecessary help	—	—	1	1
Doing transfers	—	—	1	2
Getting enough sleep	—	—	2	5
Improving dressing skills	3	6	—	—
None	—	—	—	—
Emotional Well-Being				
Talking with people	2	1	—	—
Making new friends/acquaintances	—	1	—	—
Meeting members of opposite sex	—	1	—	—
Positive self-talk regarding disability	—	1	—	—
Doing nice things for people	—	—	—	—
Getting good feelings from playing piano	1	—	1	1
Accepting group decisions	—	—	—	1
Losing weight	—	—	4	8
Not feeling homesick	1	9	—	—
None	—	—	—	2
Optional*				
Experiencing many diverse activities	2	5	1	1
Ceramics	—	1	—	—
Reading in the library	—	1	3	3
Macrame	1	1	—	—
Swimming	—	2	1	—
Fishing	—	1	—	2
Crafts	1	2	—	—
Cookout or overnight	1	1	—	1
Horseback riding	1	—	—	1
Making gifts	—	—	—	—
Learning to use better	—	—	1	1
Walking on crutches to dining hall	—	—	—	—
Playing pool	—	—	1	—
Operating ham radio	—	—	—	—
Playing baseball or football	—	—	—	—

*Subjects were allowed to choose more than one optional goal. As a result, the total number of optional goals exceeds N for each sub-group.

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ference between the two groups in any of the four areas. The only significant difference among the adults was in the area of optional goals, with the first-time campers achieving higher outcomes than those returning. Since the optional goals tended to relate to the frequency of participation in various activities, this difference indicated that the first-time adult campers tended to participate in a higher percentage of activities than they had originally planned.

A one-way analysis of variance was also done to determine if there were any differences in the level of achievement among the four goal areas for each age group. There were no significant findings for adolescents or for adults.

Discussion

Although the use of Goal Attainment Scaling to measure the outcomes of camping experience is not without problems, it appears to be feasible and appropriate. This is borne out by satisfactory GAS mean scores of 5 and 47.86. Beyond that, many campers were willing and able to participate in the process of goal-setting. Some said it was helpful, and a number enjoyed it with the interviewers.

There were a few campers, however, who did resent having to participate in the process when they arrived, expecting to begin to have fun immediately. At least one person stated that she had to strive so hard in her routine daily life that she resented the implication that she should be "achieving" at camp. Such reactions raise an important issue since the GAS procedure focuses on behavioral change; it may tend to obscure the primary purpose of a camp experience, which is simply to have fun.

In response to those campers who resented the evaluation process as conflicting with their desire for a break from the

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tion movement for handicapped children. On the other hand, the returnees knew more about the facilities available at camp. They tended to view the camp as a unique place where they could engage in many activities with

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Calsyn, R.J., Tornatzky, L.G. and Dittmar, S., "Complete Adoption of an Innovation: A Scaling..."

Professional Development

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ult Campers

GROUP

Adolescents	
1st Time (N=5)	Returning (N=10)
3	6
--	2
--	--
--	--
--	--
1	--
1	2
--	--
--	--
--	--
--	--
--	1
1	1
1	1
--	2
1	5
--	--
--	--
--	--
--	--
--	--
1	--
--	1
--	1
4	8
--	--
--	2
1	1
--	--
3	3
--	--
1	--
--	2
--	--
--	1
--	1
1	1
--	--
1	--

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the process when they arrived, expecting
to begin to have fun immediately. At
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strive so hard in her routine daily life
that she resented the implication that
she should be "achieving" at camp, too.
Such reactions raise an important point:
since the GAS procedure focuses on be-
havioral change, it may tend to overlook
the primary purpose of a camping ex-
perience, which is simply to have a good
time.

In response to those campers who saw
the evaluation process as conflicting with
their desire for a break from the pressure

to achieve, interviewers reminded them
that they were not being asked to achieve
anyone else's objectives but that this was
only a means of trying to help them at-
tain as many of the things as possible that
they personally wanted out of their
camping experience.

The interviewers also expressed con-
cern about the tendency of some indi-
viduals to be too acquiescent. Since
campers were specifically queried about
objectives in three specific areas, inter-
viewers were concerned that they might
have been creating some goals rather
than simply helping campers to identify
ones that were meaningful for them.
Nevertheless, an effort was made to
avoid this occurrence.

Another concern of the interviewers
was that some of the goals that were set
seemed unrealistic. For example, one
individual set as his "expected" level
"becoming acquainted with everyone in
the camp." As the interviewer antici-
pated, this turned out to be overly
ambitious. Some other goals were not
met simply because of limitations in
facilities or time (such as horseback-
riding slots being filled for some days
and thus being unavailable to a camper
who had set a goal of riding everyday).
However, the overall GAS scores of
50.46 and 47.86 did indicate that in the
main the goals were achievable.

The difference found between the
goal achievement of the new adult
campers and that of the returnees is not
surprising. The staff reported that first-
time campers, unfamiliar with all that is
possible for handicapped persons at the
camp, tended to minimize the types of
activities and the amount of activity that
they were capable of. This was especially
true of the older group, many of whom
live in dependent situations and have
grown up in rather sheltered environ-
ments, receiving their education prior
to the coming of the recent equal educa-

tion movement for handicapped children. On the other hand, the returnees knew more about the facilities available at camp. They tended to view the camp as a unique place where they could conveniently engage in many activities with peers that are difficult to find elsewhere. As a result, they anxiously awaited their next session and tended to look forward to participating in more frequent and more diverse activities than the first-timers—sometimes more activities than they could realistically fit into the time allotted them.

In summary, when used with proper precautions, Goal Attainment Scaling appears to be a viable tool for evaluating the impact of camping programs or the behavioral goals of participants.

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Professional 1

Exploring Therapeutic Recreation: An Ethical Response

by Charles D. Sylve

The study was conducted in their judgments of the professional role, consisting of twelve therapeutic recreation practice was the National Therapeutic subject groups by professional their level of agreement to withhold or Analysis of the data in their judgments of c Written comments im practitioners were not k uncertain of how to ma

KEY WORDS: *Professional Moral Behavior, Ethical*

Codified statements of the official moral ideology groups. Within the hel the establishment of a

Mr. Sylvester is currently study was completed as College Park, Maryland.

Third Quarter 1982

havior is a characteristic shared by the majority of human service occupations. The therapeutic recreation profession has adopted a code of ethics that subscribes equally to the Suggested Principles of the Code of Ethics of the Recreation and Park Profession and to general principles endorsed by all health related disciplines

material is a fact of life in helping systems. Thus, by virtue of the activity setting where much of therapeutic recreation occurs and the organizational system of which it is part, the therapeutic recreation profession is clearly confronted with the issue of confidentiality.

Founded on the above reasoning, this

in dealing with ethical considerable importance.

Fain has stressed the in need for ethics and has not but generally undeclared professionals regarding eth should be perceived as unstein and Compton 1977

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